

THE TRAVEL NURSE GUIDE

The Travel Nurse Playbook

What the industry does not explain, in the order you actually need it.

This guide follows the real order of a travel nursing career. Deciding whether to go, understanding how the money actually moves, getting your paperwork right, finding work, reading an offer, and handling the money side once you are on assignment.

This guide is meant to be genuinely helpful and to help you avoid the common mistakes a lot of travelers make.

READ THIS FIRST

This guide is education, not legal, tax, or financial advice. Where a topic requires a professional, it says so and points you to one.

SECTION ONE

Before You Go

Is travel nursing right for you

Travel nursing is a different job with a different set of tradeoffs. The pay can be better, but it usually comes with added risk and less stability at times. This guide is here to help you reduce that risk and decrease that instability.

Every assignment is different. Some units will hand you a light orientation and expect you to hit the ground running. Others will give you real support and a thorough onboarding. Both are possible, so go in prepared for either rather than assuming the worst case is guaranteed.

Travelers who struggle are not usually bad nurses. Sometimes it comes down to being dropped into an unfamiliar unit without enough support. Just as often it comes down to money. Traveling has real upfront costs, security deposits on housing, travel to the new location, and a gap before the first paycheck arrives. Going in without enough saved up can turn a good assignment into a stressful one fast.

THE HONEST TEST

Could you walk onto a busy unit tomorrow, in a system you have never used, and function safely without someone holding your hand? If the answer is not a confident yes, get more experience first. It will still be there in a few months.

What you need in place before you go

This is the short list. If any of these are missing, fix them before you start talking to recruiters.

READINESS CHECKLIST

- ☐ Most agencies will require at least 2 years of nursing experience and at least 1 year of experience in the specialty you plan to travel in.
- ☐ An active nursing license. It helps to know whether your license is a compact license.
- ☐ An emergency fund. Contracts get cancelled and assignments fall through. You need to be able to absorb a few weeks with no income without it becoming a crisis.
- ☐ Reliable transportation. You are responsible for getting yourself to the assignment and to work every day.
- ☐ Flexibility as a traveler. Schedules change, you may get floated unexpectedly, and you may be asked to help on an unfamiliar floor. You are there to help the facility, and being willing to help in ways you are comfortable with matters.
- ☐ A willingness to accept a challenge. Changing locations and facilities every 13 weeks, learning new systems, and caring for patients on the fly.

Know your goals and priorities before you take a single call

This section matters more than it sounds like it does.

Once you start talking to recruiters, you will have multiple agencies contacting you, each with hundreds or thousands of open jobs. Without a filter, that volume is paralyzing. Every option looks reasonable and you end up choosing based on whoever called most recently rather than what you actually want.

Your goal is the filter. Most travelers are chasing one of three things:

- **Money.** You are here to pay off debt, build savings, or buy something. Highest rate wins, and you will tolerate a rough assignment or an undesirable location to get it.
- **Adventure.** You want to live somewhere you have never lived. Location drives the decision and you will accept a lower rate to be where you want to be.
- **Career.** You want a specific kind of experience. A level 1 trauma center, a particular EMR, a certain patient population, something that makes you better or makes your resume stronger.

Most people are some mix of all three. Rank them. Knowing your priority order helps you filter fast. When a recruiter sends you 12 jobs, you will know instantly which 2 are worth a conversation.

The tradeoff nobody names

Here is a question worth answering honestly before you start.

Would you take the highest offer available even if it comes from a recruiter you are not sure you trust? Or would you take somewhat less to work with someone you know is straight with you?

There is no wrong answer, but you should know yours. In a survey of TNG members, roughly 1 in 3 said they would accept about \$100 a week less to keep working with a recruiter they trusted.

That number is worth sitting with. For a lot of travelers, it usually comes down to simplicity, consistency, and the value of a long term, trustworthy relationship over an extra \$1,300 across a 13 week contract. To each their own.

SECTION TWO

How The Money Actually Moves

A lot of travelers make costly mistakes because they do not understand this dynamic.

The bill rate

When a hospital needs a traveler, it agrees to pay an agency a set amount per hour for that nurse. That amount is called the bill rate. It is negotiated between the hospital and the agency before you are ever in the picture.

The agency takes that bill rate and uses it to cover everything on its side. Overhead, your benefits, insurance and liability, the recruiter's commission, and profit. Whatever is left is the maximum it can offer you. That does not mean it is what you will be offered. Agencies and recruiters will sometimes offer less than that maximum to increase their own margin.

This is also what negotiating actually is. You are asking the agency for a higher portion of the bill rate. You are not negotiating with the hospital, and the bill rate itself does not change.

HOSPITAL	AGENCY	YOU
Sets the bill rate	Keeps a portion for costs and profit	Receive the remainder as a pay package

This is why two nurses can work the same unit, on the same shift, at the same hospital, through two different agencies, and take home very different money. Same bill rate. Different split.

What you are actually negotiating

When you negotiate your pay, you are not asking the hospital to pay more. The hospital is not in the conversation and its bill rate is already set. You are asking the agency to keep less of it.

THE CORE TRUTH

In its most basic form: the less you make, the more the agency makes. The more you make, the less the agency makes. That is the entire dynamic, and it is why you should never assume the first number you are offered is the best one available.

This does not make agencies villains. They have real costs and they are entitled to a margin. But it does mean the person offering you a rate has a direct financial interest in you accepting a lower one, and you should evaluate their number accordingly.

MSPs and why the same job pays differently

Many hospitals do not manage their own travel staffing. They contract it out to a Managed Service Provider, or MSP. The MSP handles the hospital's traveler needs, distributes open jobs to other agencies, and manages the process.

Here is why that matters to your paycheck. If you work at that facility through an agency that is not the MSP, your agency typically has to pay the MSP a fee, commonly somewhere in the range of 5 to 10 percent, for access to the contract. That fee comes out of the same bill rate your pay package comes from.

Sometimes going through the MSP directly means more of the bill rate is available for you. Sometimes it does not, because agencies differ in how much margin they take. It is worth asking whether the agency you are talking to is the MSP for that facility.

What to understand about your recruiter

You owe your recruiter nothing. Not loyalty, not exclusivity, and not the benefit of the doubt.

A recruiter can be genuinely kind, responsive, and pleasant to work with, and still be structurally incentivized to pay you less. Those two things are not in conflict. Being nice is not the same as being on your side.

There are excellent recruiters in this industry who will tell you when a contract is bad for you and talk you out of it. There are also recruiters who will let you take a below market rate and never mention it. It is difficult to tell which one you have until you are already in a contract, and by then it could be too late.

One resource that can help: GreatRecruiters.com lets travelers see recruiter and agency reviews from real nurses before they commit.

A recruiter's job is to fill the contract. Your job is to make sure the contract works for you.

SECTION THREE

Getting Ready

Build your document vault

Every agency you work with will ask for essentially the same documents. If you are working with three agencies, you will upload the same license three times. If you do not have these organized, onboarding becomes the most frustrating part of the whole process.

Save all of these in one folder, on your phone and backed up somewhere, so you can attach or upload any of them in under a minute.

DOCUMENTS TO HAVE SAVED AND READY

- ☐ Nursing license or licenses
- ☐ Driver's license, front and back
- ☐ Social Security card
- ☐ All current certifications with visible expiration dates
- ☐ Immunization records, including titers
- ☐ Most recent TB test or chest x-ray
- ☐ Most recent physical
- ☐ Flu shot record, seasonally required by most facilities
- ☐ COVID vaccination record if applicable
- ☐ Your travel resume, updated
- ☐ Two professional references with current contact information

On references. Most agencies require two references from people you worked with in the past 12 months, usually a charge nurse or manager. This is more annoying than it sounds. Your reference may work nights while the recruiter calls during the day, and the two of them can miss each other for a week while your submission sits.

Give your references a heads up before you list them, tell them who is calling and roughly when, and confirm the best number and time to reach them. It will save you time, especially when you are trying to quickly submit for a good contract.

Certifications

An expired certification will delay or kill a start date, and it is one of the most avoidable problems in travel nursing. Certifications generally run 2 years, and they always seem to expire at the worst possible moment.

Know what your specialty requires and what each facility may require on top of that. Common ones include BLS, ACLS, PALS, NRP, TNCC, ENPC, and CPI. Your recruiter will tell you which ones are required for the specific facility and unit you are traveling to.

Two things worth knowing about renewals:

- Most facilities require BLS, ACLS, and PALS specifically through the American Heart Association. If these three are required, get them through AHA.
- AHA Verification Stations remove the classroom portion of the certification. You complete the online cognitive portion at home, then find a verification station near you to complete the hands on portion with a voice assisted manikin. If you pass, your certificate prints instantly and you are certified on the spot.

Put every certification expiration date in your phone calendar with a 90 day reminder, today. This single habit prevents one of the most common and most costly scheduling problems travelers run into.

Your travel resume

A travel nursing resume is not a normal resume, and treating it like one makes it harder for you and your recruiter to get your information submitted to the facilities you want.

A traditional resume is one page and summarizes. A travel resume is as long as it needs to be, often 2 to 3 pages, and lists every assignment in reverse chronological order with the same specific details for each one.

Every contract on your resume should list:

- Facility name, city, and state
- Unit and your role
- Start and end dates
- Whether it was a travel contract, and the agency if so
- Facility type and whether it is a teaching facility
- Trauma level
- Total facility beds, and beds on your unit
- Typical patient ratio or case load
- EMR system used

COMING SOON

A free travel resume builder is coming to travelnurseguide.com. It will format all of this correctly and let you add each new contract as you finish it, so your resume is always current without rebuilding it every time.

SECTION FOUR

Who Should You Work With

Finding work

The common advice is to work with more than one agency, and that is still good advice for most travelers. But it is not the only valid approach. Some travelers get real value out of loyalty and consistency with one great recruiter, even if it occasionally means slightly less pay. Others prefer working with multiple agencies and always pushing for the highest offer on every contract. Both are legitimate. Know which one fits you.

No agency has access to every job. No agency offers the best rate on every contract. If you only talk to one, you have no idea whether the offer in front of you is good, because you have nothing to compare it to.

Even if you choose to work with one recruiter, keep an eye on other agencies, job boards, and Facebook groups so you know what a good contract looks like in your specialty and area. It is your recruiter's job to find you good contracts. It is your job to verify it.

A structure that works well for a lot of travelers:

- **1 to 2 large MSP agencies.** They hold or manage a high volume of contracts and give you broad visibility into what is available.
- **1 to 2 agencies you actually like.** Maybe the recruiter is genuinely good. Maybe they have direct relationships with specific hospitals in your home state that you want to work at. Maybe their benefits fit your situation better.

There is no cap. Some travelers work with 3 to 4, some work with more. Work with as many as makes sense for you.

Some agencies will try to get you to complete their full application before they will discuss pay. This is often a tactic to collect your information and make you feel invested before you have real numbers. You can push back and ask for pay details first. Most will still talk numbers with you.

Research the market before you talk rates

Walk into the conversation already knowing what the market looks like. If you do not have a benchmark, you cannot recognize a lowball when you hear one.

Look at posted jobs for your specialty in the states and cities you are considering. Useful places to check include Facebook travel nurse job groups, online job boards, agency websites, and agency email lists.

If a rate is posted, or a recruiter sends you a specific rate in writing, hold them to it.

Research housing before you accept, not after

Have a clear idea of housing availability and cost before you accept a contract. Very expensive or nonexistent housing in that area can turn a good contract into a bad or unprofitable one.

Before you accept, confirm two things: that furnished monthly housing exists in that area at all, and roughly what it costs.

Submitting: a big rule to follow

Try to avoid having two different agencies submit you to the same facility at the same time.

This applies within the same job search cycle, not forever. If you submit through Agency A to Hospital B in the spring, and later submit through Agency C to Hospital B in the fall, that is fine. The issue is having more than one active application for you sitting in the same stack at the same hospital at the same time.

A manager who finds two applications with your name on them often does not want to sort out which agency you actually want to work with. In practice, a lot of facilities simply throw both applications out rather than deal with it. Duplicate active submissions will not damage your standing, but they can meaningfully decrease your chances of getting that offer.

The fix is simple and entirely on you. Keep a written record of which agency submitted you to which facility and when. A note on your phone is enough. Check it before you approve any new submission.

The phone interview

These calls are often short and low pressure. Most of the time, the manager has already reviewed your profile and application and is close to offering you the job. The call is just as much your opportunity to ask questions and get a feel for whether this is a place you want to work.

Managers are always watching for red flags, so it helps to be energetic, positive, and appreciative during the call. Keep your phone nearby while you are waiting on an interview. Missing the call can sometimes mean missing the contract.

Have your own questions ready. The list below is what I actually recommend asking.

QUESTIONS TO ASK DURING THE PHONE INTERVIEW

- ☐ Orientation process?
- ☐ Charting system?
- ☐ Nurse to patient ratio?
- ☐ Are there techs and other ancillary staff?
- ☐ Are there other travelers in the department? How many?
- ☐ Will I be floated to other floors?
- ☐ Will I be called off if census is low?
- ☐ How is scheduling handled? Block schedule? Self schedule? Holidays? On call?
- ☐ On call requirements?
- ☐ Am I allowed to work overtime?
- ☐ Uniform requirement? Scrub colors? Do I have to cover my tattoos?
- ☐ Average experience level of other nurses in the department?
- ☐ Common patient demographics?
- ☐ Is English the main language spoken by patients?
- ☐ Parking? Free or paid?

SAY THIS OUT LOUD

You do not have to accept and sign the contract just because they offered it. An offer is not an obligation. If the interview raised concerns, if the ratios are worse than described, if the float expectations changed, if you have already been offered a better contract with a different agency, or if you have simply changed your mind about the location, you are allowed to decline.

SECTION FIVE

The Offer

You should have a full pay package in hand, negotiated to a satisfactory rate, before you submit. If a recruiter cannot get the pay package to a rate you are satisfied with, there is no reason to submit for that contract.

Reading a pay package

A travel pay package usually includes a taxable hourly rate and tax free stipends, and may also include reimbursements, bonuses, an overtime rate, and an on call rate. Look at every piece carefully. If something is not listed in the pay package or contract, it is not guaranteed to happen.

- **Taxable base rate.** Some nurses try to shift more of the package into stipends and less into the hourly rate. It is common to see taxable rates between \$20 and \$30 an hour. On a high paying contract with stipends already maxed out, the hourly rate may need to be higher.
- **Housing stipend.** Tax free if you qualify.
- **Meals and incidentals.** Also tax free if you qualify.
- **Overtime rate.** Overtime is not always calculated off your taxable base alone. I recommend negotiating your overtime rate to 1.5 times your blended rate. Some agencies will try to pay 1.5 times the hourly rate only, which is low. If that is the structure you are offered, it is often not worth working overtime.

Always compare gross weekly, itemized. Ask every recruiter for the same thing: gross weekly pay with each component broken out separately. Do not compare take home estimates, since those are the recruiter's assumptions about your taxes, not a guarantee.

Two ceilings worth knowing. The GSA publishes maximum tax free stipend amounts by location, and agencies cannot exceed them. And an unusually low taxable base rate, well below market for your role, can draw IRS scrutiny. Many experienced travelers and tax professionals treat a base below \$20 an hour for an RN as a flag worth asking about.

Verify the contract before you sign

This is the highest value checklist in this guide. A verbal promise from a recruiter is worth nothing. If it is not in the written contract, it does not exist.

Read the actual document, not the offer summary email. Confirm every line below.

CONTRACT VERIFICATION CHECKLIST

- ☐ Taxable hourly rate, exactly as quoted
- ☐ Housing stipend amount and how it is paid
- ☐ Meals and incidentals amount
- ☐ Guaranteed hours per week, and what happens if you are not given them
- ☐ Overtime rate and at what threshold it begins
- ☐ Holiday rate, and specifically which days the facility counts as holidays
- ☐ On call rate and callback rate, if on call is required
- ☐ Call off and low census clause. How many shifts can they cancel, and are you paid?
- ☐ Float clause. Which units, and are they limited to areas you are competent in?
- ☐ Cancellation policy on both sides. Notice required, and any penalty to you
- ☐ Start and end dates, and the exact number of weeks
- ☐ Shift, hours per shift, and scheduled days
- ☐ Travel reimbursement, if promised
- ☐ License or certification reimbursement, if promised
- ☐ Any bonus, the exact conditions to earn it, and when it pays out

THE RULE

If a recruiter told you something on the phone that you do not see in the contract, ask for it to be added in writing before you sign. If they will not put it in the contract, treat it as something that is not going to happen.

A word on leverage

Negotiating is its own subject and this guide will not try to cover it fully. But it is worth knowing what leverage actually is, because most travelers think they have none.

Your leverage is information and alternatives. Knowing the market range for your specialty in that city is leverage. Having a competing offer from another agency is leverage. Being genuinely willing to walk away is leverage.

What is not leverage is asking for more because you would like more. If you cannot point to something concrete, you are asking the agency to voluntarily reduce its own margin for no reason.

SECTION SIX

The Money Side

Taxes and your tax home

There are 3 requirements you must meet, all of them, to qualify for tax free stipends under IRS tax law. Agencies will sometimes pay stipends tax free without verifying whether a traveler actually meets those requirements, in part because doing so saves the agency roughly 7 percent in payroll taxes. Just because an agency is willing to pay you tax free does not mean you actually qualify. It is your responsibility to verify your own qualification.

- **Maintaining a tax home.** A residence you pay to keep and return to between assignments.
- **Duplicating expenses.** You are genuinely paying for housing in two places at once, at fair market value.
- **Distance.** The assignment must be far enough from your tax home to reasonably require you to rest away from home.

This is not tax advice. Consult a tax professional to confirm your specific situation.

One myth worth correcting: the IRS does not publish a 50 mile rule. That number is an internal threshold many agencies use to decide when they are comfortable paying stipends.

WHERE TO GET REAL ANSWERS

This guide will not go deeper than that, deliberately. Tax home qualification is genuinely complicated and depends on your specific situation, and getting it wrong could be expensive if audited by the IRS. Work with a tax professional who specializes in travel healthcare.

Keep documentation as you go. Leases, rent receipts, mortgage statements, utility bills, and proof of payment for both your tax home and your assignment housing. If you are ever audited, this paperwork is the entire case.

Insurance: agency or private

Most travelers take the agency plan by default because it is sitting in front of them during onboarding. It is worth understanding the actual tradeoff.

AGENCY INSURANCE	PRIVATE INSURANCE
PROS • Convenient, already part of onboarding • Agency typically covers part of the premium, though that money comes from the bill rate, meaning it is effectively money that would otherwise go to you • Usually no underwriting or approval required	PROS • Not tied to your contract or employment • No coverage gaps between assignments • Deductible does not reset when you switch agencies • Freedom to take extended time off • Can be more affordable for some travelers • Some plans cover you in all 50 states, though coverage depends on the specific plan • You may be able to negotiate to have the insurance portion added back into your pay package if you opt out of agency insurance
CONS • The biggest issue is gaps in coverage between contracts, especially if a contract is cancelled • Deductible restarts from zero when you change agencies	CONS • You have to research which private option is best for you

Insurance rules change by state and by plan year, and travel nurses have a situation most brokers rarely see.

Housing and how travelers get burned

Housing is one of the most stressful recurring tasks in travel nursing, and the one where travelers lose the most money to outright fraud. Be aware of scammers. Below is a list of things that can help you avoid getting scammed.

Never send money before you verify the property exists and the person you are talking to controls it. Not a deposit, not first month's rent, not an application fee. See it in person, or do a live video walkthrough with the actual landlord where they show you the unit in real time.

WALK AWAY IMMEDIATELY IF

- ☐ They pressure you to pay today or lose the unit. Urgency is the primary scam tool.
- ☐ They ask for payment by wire, Zelle, Venmo, Cash App, or gift card before any tour
- ☐ The price is dramatically below every comparable listing in that area
- ☐ The landlord is conveniently out of town and cannot show it or do a live video
- ☐ The name on the lease does not match the person you have been talking to
- ☐ They refuse to provide a written lease

Get a lease and keep proof of every payment. Beyond protecting you as a tenant, a signed lease and documented rent payments are among the most important records you can produce if you are ever audited on your stipends. Pay by check or bank transfer so there is a trail.

SECTION SEVEN

Vetted Resources

This guide points you to a professional in a few places. Tax home qualification, private insurance, certifications, and housing all have people who specialize in travelers, and finding them on your own is harder than it should be.

Every resource on the site has been personally reviewed. This is not an open marketplace, and nothing appears there because someone paid to be listed and nothing more. Some are affiliate links, which means TNG may earn a commission if you use them. That never changes what you pay, and it never determines who makes the list.

The list lives on the website rather than in this guide for one reason. Resources change, and a PDF sitting in your inbox does not. The page is always current.

EVERYTHING IN ONE PLACE

Vetted resources for travelers

Tax professionals, insurance, certifications, housing, and AHA verification stations.

travelnurseguide.com/resources

ONE LAST THING

The community is where the rest of this happens

37,000 travel nurses answering each other's questions honestly, every day. Somebody in there has already lived your next contract.

travelnurseguide.com

